



OFFICIAL

NMHS MHS PROCEDURE

Release of Medical or Psychiatric Information under the Mental Health Act 2014 or Freedom of Information Act 1992

Scope (Audience)	All NMHS Mental Health Services (MHS) & Freedom of Information (FOI) Staff
Scope (Area)	All NMHS MHS Freedom of Information Staff
Summary	To outline the process for the release of medical and/or psychiatric information under Section 248 and 249 of the Mental Health Act 2014 (MHA 2014) and the Freedom of Information Act 1992 (FOI Act)
Risk Rating	Medium
NSQHS Standards	Clinical Governance (Std 1) Partnering with Consumers (Std 2)
Policy Sponsor	Director of Strategic Projects MHSDHS
Policy Contact	Area Health Information Manager/Area FOI Coordinator MHS
Version	V1.0
Review Date	31/10/2028

COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

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1. Aim

This procedure outlines the process for the release of medical and/or psychiatric information under Section 248 and 249 of the [Mental Health Act 2014](#) (MHA 2014) and the [Freedom of Information Act 1992](#) (FOI Act).

2. Risk

Non-compliance with this policy will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☐
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

3. Requests for Information under the MHA 2014

The MHA 2014 s.248 supports a person's rights to inspect, and to be given a copy of, any relevant document relating to the person.

Importantly there are exceptions (s.249) excerpt:

Section 249 Restrictions on access

- (1) *A person is not entitled to have access under section 248(1) to a relevant document relating to the person:*
- (a) *If a psychiatrist reasonably believes that disclosure of the information in the document to the person –*
- (i) *poses a significant risk to the health or safety of the person or to the safety of another person;*
- (ii) *poses a significant risk of serious harm to the person or another person.*
- or*
- (b) *If disclosure of the information in the document to the person would reveal-*
- (i) *personal information about an individual who is not the person; or*
- (ii) *information of a confidential nature that was obtained in confidence.*

3.1 Process

Opinions prior to the provision of documents will now only be sought when s.249 and s.250 may be applicable.

Section 249 may be applicable when:

- A clinician has identified a **significant risk** (to the applicant or others) and logged a PSOLIS Alert, and a copy has been placed in the patient's file.
- The Freedom of Information (FOI) Coordinator has identified a potential risk and has considered the following:

- Frequency of FOI request;
- PSOLIS Alert noting suicidal ideation, suicide attempt or other vulnerability requiring greater input by the clinical team; and
- Mental Health Act status.

Under Section 249 a significant risk is when the psychiatrist reasonably believes that disclosure of the information “poses a significant risk to the health or safety of; or of serious harm to the person or another person”.

When an opinion is sought, a response from the clinician is required within two (2) weeks.

If the opinion is not provided within the prescribed time, the proposed document will be released in accordance with the MHA 2014.

Note: Prior to invoking the provisions of s.249 of the MHA 2014 the FOI Coordinator will seek an opinion from the treating Consultant Psychiatrist. The FOI Coordinator makes the final decision on access.

3.2 Communication/Notification of decision

The FOI Coordinator will send out a Notification of Decision letter to the applicant and where access is approved, include a copy of the documents being released as part of the application.

3.3 Applications under s.248 of MHA 2014 by a Guardian

Under s.248 of the MHA 2014, only the person who has been provided treatment or care by a mental health service is entitled to inspect or be given a copy of the documents that are under the control of the mental health service. If a legal guardian applies for access to medical or psychiatric information under the MHA 2014 they should be directed to apply for access under the Freedom of Information Act 1992. For further information on release of patient information to a guardian please consult the [NMHS MH Release of Patient Information Policy](#).

4. Request for Information under Freedom of Information Act 1992

Section 28 of the FOI Act 1992 states:

Medical and psychiatric information about applicant, giving access to

If:

- (a) a document to which the agency has decided to give access contains information of a medical or psychiatric nature concerning the applicant; and*
- (b) the principal officer of the agency is of the opinion that disclosure of the information to the applicant may have a substantial adverse effect on the physical or mental health of the applicant.*

4.1 Process

- If the Freedom of Information Officer considers there may be a risk of harm to the applicant a clinician’s opinion will be sought as to whether the release of information may have a substantial adverse effect on the applicant.
- A clinician (the current active senior doctor or the Medical Co-Director) of the service the consumer had either been active or engaged with is required to provide an opinion within two (2) weeks.
- The clinician provides their advice on release by return email.

- FOI Coordinator makes the decision on access based on the clinician's advice.
- The FOI Coordinator will notify the clinician after two weeks of no reply that the documents are about to be released.
- Should no opinion be provided in the prescribed time, the proposed documents will be released in accordance with the FOI Act.

4.2 Personal information of staff

- Staff member's names, qualifications, position, functions/duties of the person, anything done by the person in the course of performing his or her duties are **not** exempt: see Schedule 1, Clause 3(1)(3) and (4) of the FOI Act and also regulation 9 of the [Freedom of Information Regulations 1993](#).
- Ask the applicant if the scope of the FOI request can be limited to exclude staff names (and qualifications etc) so they can be redacted by agreement.
- If the applicant does not agree to limiting the scope to exclude staff names (and qualifications etc), FOI must not give access until they have taken such steps that are reasonably practicable to obtain the views of the third party (i.e. the staff member)

Note: Prior to invoking the provisions of s.28 of the FOI Act, the FOI Coordinator will seek endorsement by the Principal officer, the Chief Executive of the Health Service Provider.

5. Monitoring and Evaluation

Compliance with this procedure is mandatory. There are legislative obligations relating to this procedure where a statutory penalty may apply for non-compliance.

Clinicians to provide feedback in respect to process efficiency, clinical workflow and outcome.

This procedure will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed 3 years using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

6. References

- [Freedom of Information Act 1992 \(Western Australia\)](#)
- [Mental Health Act 2014 \(Western Australia\)](#)

7. Related Documents

Department of Health Policy documents	• MP 0015/16 Information Access, Use and Disclosure Policy • Department of Health Legal and Legislative Services – Patient Confidentiality
NMHS Policy documents	• NMHS Information Management: Management of Freedom of Information (FOI) Applications Policy • NMHS Information Breach Response Procedure • NMHS Healthcare Record Access, Use and Disclosure Policy • NMHS Healthcare Record Management Policy
NMHS MHS Policy documents	• MHS Release of Patient Information Policy • MHS Patient Confidentiality Guideline

Other	• Office of the Information Commissioner (OCI) – Information Resources for agencies • Office of the Information Commissioner (OCI) – FOI Coordinator's Manual
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8. Document Control

Document Version Control			
Date	Version	Author	Notes
31/10/2025	V1.0	Area Information Manager MHSDHS	Initial endorsement

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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